

Eastern UP Multi-Site Hospital Platform

Indicative investor discussion pack | anonymized management data and external market references

Current net revenue Rs 49.2 cr	Current EBITDA Rs 16.8 cr	EBITDA margin 34.2%	Current PAT Rs 8.4 cr
Installed capacity 225 beds	Blended occupancy 28.3%	ARPOB / RPOB Rs 26,000	ALOS 3.8 days

Transaction perimeter

Full-platform sale comprising the operating hospital business, the owned primary campus real estate, and the secondary leased campus operations. Optional expansion land is discussed later and is not part of the opening page summary.

Current platform read-through

Current reported EBITDA of Rs 16.8 cr and margin of 34.2% are generated on a platform that still carries underutilized capacity. ARPOB is already at Rs 26,000, suggesting monetization quality on current admissions rather than purely volume-led economics.

Owned campus as downside support

Owned primary campus reference is approximately Rs 52.4 cr. At a full-platform discussion band of roughly Rs 250-Rs 275 cr, this represents about 19.1% to 21.0% of total platform value, providing a meaningful hard-asset layer in a downside or dislocation scenario.

What is intentionally not crowded into page 1

Detailed inter-company elimination schedules, equipment-level diagnostics detail, selective uplift capex, and optional land parcels are kept for later pages so the opening investor read remains clean and not over-explained.

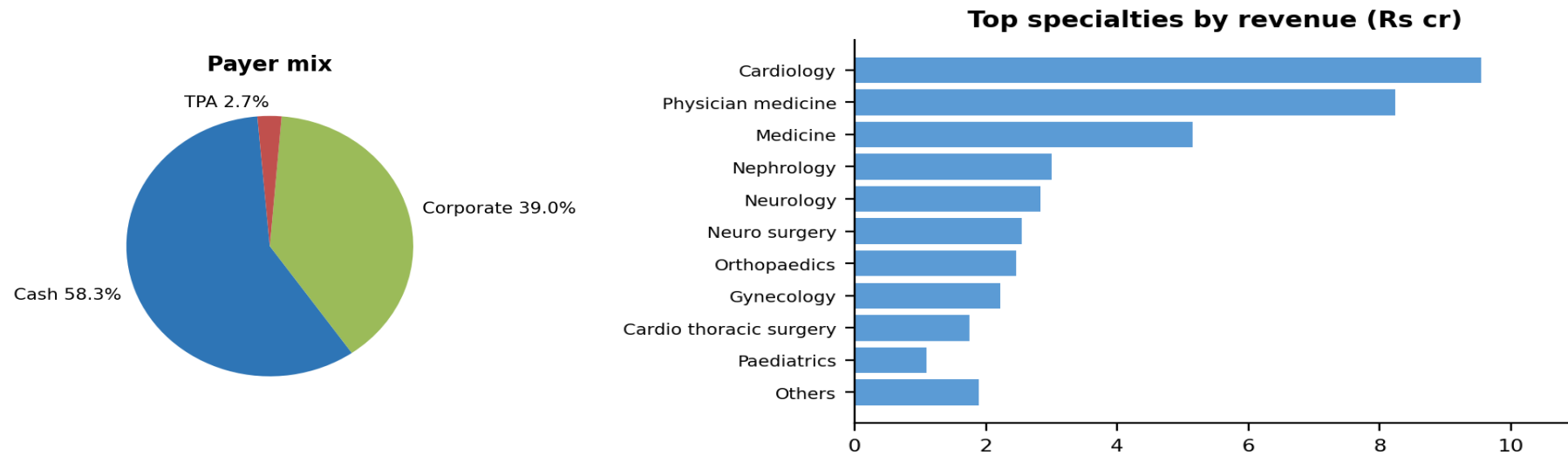
1. Platform overview and operating index dashboard

Site	Status	Land area	Built-up area	Beds	Avg occupancy	Current note
Primary owned campus	Owned	16,200 sq ft	38,000 sq ft	125	35%	Owned flagship campus with hard-asset backing.
Secondary leased campus	Leased	8,000 sq ft	27,000 sq ft	100	20%	Institutional catchment node; PET-scan insertion space available.

Average occupied beds 63.8	Occupied bed-days 23,269	Estimated discharges 6,123
BTR 27.2	BTI 9.6 days	PAT margin 17.0%

Current platform KPI	Current level	Interpretation for an investor
ARPOB / RPOB	Rs 26,000	Current monetization benchmark is already high for the region and has moved up from roughly Rs 25k in the latest normalized year.
ALOS	3.8 days	Within a reasonable multispecialty range and supportive of bed turnover.
Occupancy	28.3% on 225 beds	Below mature-chain occupancy; this is why underutilized infrastructure remains a large part of the upside story.
IPD / OPD+daycare split	50.7% / 49.3%	Balanced clinical engine rather than over-reliance on one revenue stream.
Cash + corporate mix	97.3%	Low TPA dependence reduces adjudication drag and supports collection quality.

2. Revenue quality, specialty mix and why the current mix matters

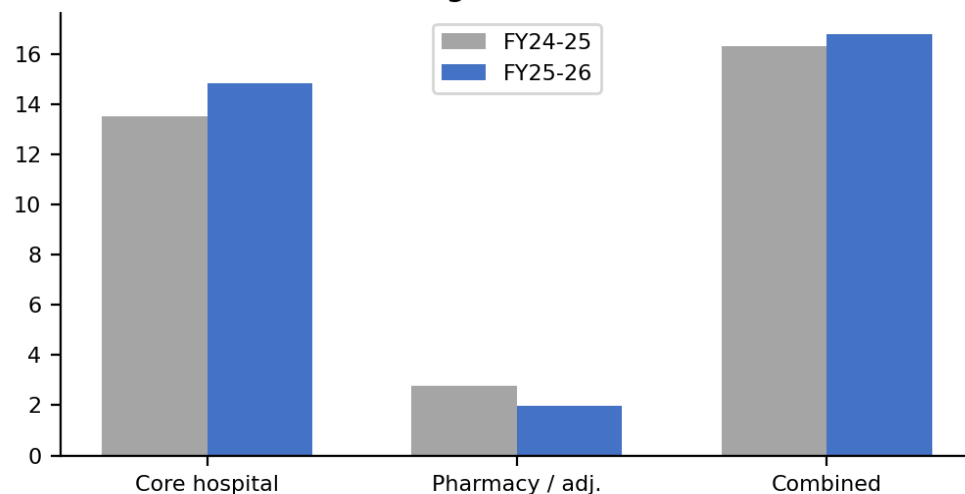


Mix indicator	Current level	Why it matters
Payer mix	Cash 58.3% Corporate 39.0% TPA 2.7%	A cash and corporate skew reduces dependence on slower TPA adjudication and can support stronger collection quality.
Net revenue mix	Patient services 82.3% Pharmacy 17.7%	Pharmacy is meaningful without dominating the business; core clinical revenue remains the main earnings driver.
Patient revenue mix	IPD 50.7% OPD+Daycare 49.3%	A balanced inpatient / outpatient profile improves resilience and gives multiple touchpoints for diagnostics, surgeries, and follow-up care.
Specialty concentration	Top 8 specialties ~85.5% of specialty revenue	Cardiology, physician medicine, medicine, nephrology, neuro sciences, ortho and gynecology create a broad tertiary base rather than one narrow specialty bet.
Oncology	Currently low base	Low current base creates case-mix headroom; in this pack, oncology is presented as phased upside, not the core underwriting base.

This mix combines bread-and-butter volume specialties with higher-acuity departments, while collections remain heavily weighted to cash and corporate billing. That supports cash generation and limits dependence on one specialty or one payer bucket.

3. Historical performance, normalization bridge and inter-company note

Normalized EBITDA growth (FY24-25 to FY25-26)



Normalized growth reading

FY23-24 is excluded from the normalization bridge because management indicated a non-recurring COVID / government project contribution in that year. On the cleaner FY24-25 to FY25-26 base, core hospital EBITDA grew by 9.9%, ARPOB by 4.0%, and occupancy by 14.3%.

Metric	FY24-25	FY25-26	Normalized growth	Reading
Core hospital EBITDA	Rs 13.51 cr	Rs 14.85 cr	+9.9%	Most relevant single growth line for hospital buyers.
Pharmacy / adj. EBITDA	Rs 2.79 cr	Rs 1.96 cr	-29.8%	Smaller and more volatile than the hospital entity.
Combined EBITDA	Rs 16.30 cr	Rs 16.81 cr	+3.1%	Combined reported growth remains positive.
ARPOB	Rs 25,000	Rs 26,000	+4.0%	Pricing / case-mix moved up year-on-year.
Management occupancy trend	35%	40%	+14.3%	Volume growth complements pricing improvement.
Internal pharmacy transfer	Rs 3.25 cr	Rs 3.60 cr	+10.8%	Disclosed separately for diligence; elimination schedule can be provided.

Inter-company note: pharmacy billings have been disclosed because there is cross-hold and a linked pharmacy channel. Since the transfer value is known but the transfer-margin detail is not separately broken out here, this pack discloses the amount and avoids a mechanical EBITDA adjustment in the teaser itself.

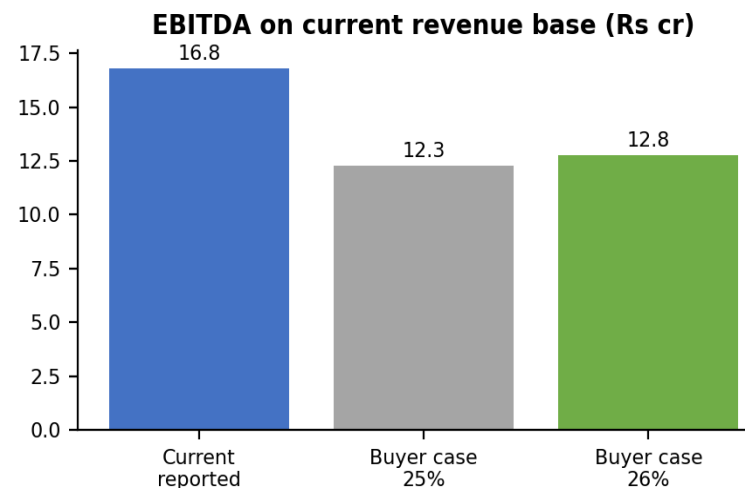
4. Current EBITDA margin - what can support it and how to read it

What can support a high current EBITDA margin

1) Heavily cash + corporate mix with only limited TPA dependence. 2) Integrated diagnostics capture, including two in-house diagnostic centers with MRI capability. 3) Pharmacy attachment and internal clinical capture. 4) Tier-2 operating base with structurally lower overhead than large metro hospitals. 5) ARPOB already elevated for the region. 6) A large part of the core infrastructure is already in place.

How low occupancy but high ARPOB should be read

Low occupancy here should be read as underutilized infrastructure rather than weak monetization. On current admissions, the platform is already monetizing well at roughly Rs 26,000 ARPOB. That combination often indicates operating leverage if the platform scales with the right clinicians, marketing, and selective OPEX additions.



Infrastructure-ready and selective uplift capex

The platform does not require a foundational rebuild. Management indicates that a selective readiness / uplift capex of approximately Rs 5.0 cr can support refurbishment, equipment refresh, activation, and productivity improvement. This is best described as acceleration capex rather than rescue capex.

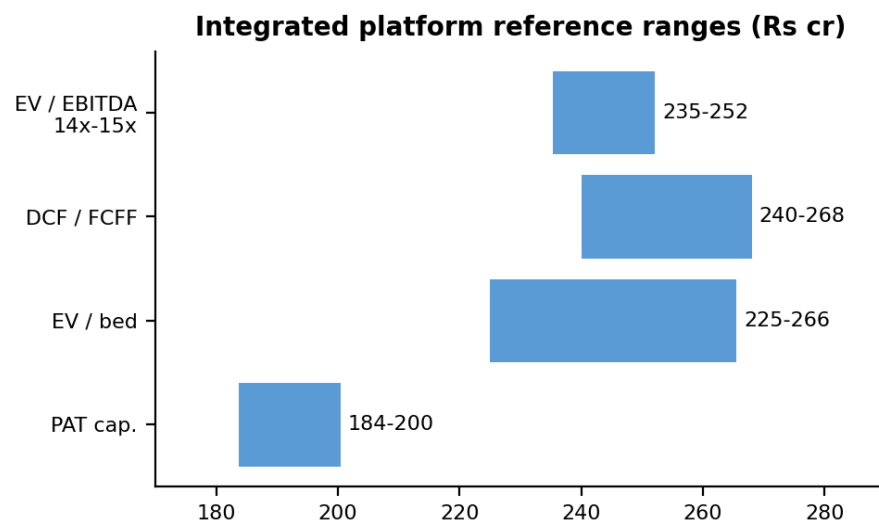
Buyer-case underwriting can therefore be kept at 25%-26% EBITDA margin for conservatism, even though current reported margin is higher. That makes the current margin a source of internal cash-generation headroom rather than the only basis of the forward case.

5. Extracted external market references and how they frame the platform

Reference source	Extracted highlight	Why it matters here
ICRA hospital industry outlook (Dec 2025 / Jul 2025)	Sample-set occupancy around 62%-64% in FY26-FY27; operating margins around 22%-24%; revenue growth 18%-20% expected in FY27.	Current platform occupancy is below mature-chain benchmarks, but current margin is already above broad-sector OPM levels.
CareEdge hospital sector note (Dec 2025)	Indian hospital sector growth projected around 10%-12% CAGR over the next 2-3 years; EBITDA margins stabilised near 21%-22%.	A 25%-26% buyer-case EBITDA margin is conservative relative to current reported performance.
Apollo Hospitals FY25 investor update	Healthcare Services occupancy around 67%; ARPOB per day around Rs 63,569; EBITDA margin around 24.3%.	Provides a mature-chain operating benchmark for occupancy, ARPOB and margin; directional, not like-for-like.
Listed hospital sector commentary (Dec 2025)	Listed hospital coverage discussed broadly around 20x-25x forward EV / EBITDA.	Listed benchmarks sit higher; private single-platform discussions usually apply a discount.
North India strategic transaction reference (Sep 2025)	A 150-bed hospital transaction implied roughly Rs 1.73 cr per bed.	Supports the view that selected North India strategic transactions can reach strong per-bed values.
NABH Hospital Accreditation Standards, 6th edition (Jan 2025)	Current external quality framework for governance, clinical process, patient safety and facility systems.	Useful quality / governance reference point for institutional buyers and lenders.
ICRA bed addition note (Feb 2026)	Large chains expected to add >34,000 beds over FY26-FY30 with ~Rs 40,000 crore cumulative capex.	Supports replacement-cost logic and bed-capex economics for growth and rebuild scenarios.

Taken together, these references place the platform in a favorable discussion zone: buyer-case underwriting remains below current reported margin, while bed-replacement and selected transaction signals provide a separate asset-backed cross-check.

6. Integrated-platform valuation references and asset-light option



How this page should be read

The ranges on this page are presented as integrated-platform references. In other words, the owned primary campus is treated as part of the operating platform context and is not added back separately onto the EBITDA multiple. This avoids the double-counting concern that can arise when owned operating real estate is bolted on mechanically to a business multiple.

Method	Basis used	Integrated platform reference (Rs cr)	Comment
EV / EBITDA	14.0x-15.0x current EBITDA of Rs 16.8 cr	235.3 - 252.1	Headline multiple discussion for a full-platform operating acquisition.
DCF / FCFF	5-year ramp and 25%-26% buyer-case margin	240.0 - 268.0	Captures embedded operating leverage and case-mix improvement.
EV / bed / replacement logic	Rs 1.00-Rs 1.18 cr per installed bed on 225 beds	225.0 - 265.5	Cross-check rooted in platform capacity and replacement economics.
PAT capitalization	22x-24x PAT	183.7 - 200.4	Useful conservative cross-check, but not the only valuation lens where embedded capacity and owned campus matter.
Main discussion band	Weighted toward EBITDA, DCF and EV / bed	234.4 - 261.0	Investor conversation can usually sit here more naturally than on the most conservative PAT lens alone.

Asset-light option (kept as a footnote, not the headline)

If a buyer prefers a lower upfront capital commitment, an asset-light discussion can also be explored by deducting the owned campus reference of about Rs 52.4 cr from the integrated-platform discussion band and then normalizing a long-term lease. On that basis, the indicative asset-light discussion range would be approximately Rs 182.0 - Rs 208.6, subject to lease structuring and final commercial terms.

7. Internal cash generation, readiness capex and optionality beyond the base deal

Company debt structure

Company debt considered in the operating bridge is Rs 25.0 cr. This pack focuses on the business and platform discussion rather than a detailed debt waterfall, which would be more appropriate for confirmatory diligence.

Post-cleanup annual cash retention

An annual principal release of roughly Rs 1.8 cr is not treated as EBITDA, but it can support faster debt reduction, working capital, equipment refresh, and productivity-led reinvestment.

Infrastructure ready; revenue can scale with the right OPEX

A significant portion of the clinical and physical platform is already in place. The main unlock is execution: clinician additions, better throughput, referral conversion, selective diagnostics attachment, and sharper commercial management rather than a full rebuild.

Not in the base deal / separate optionality

Additional land parcels and incremental bed options can be discussed separately. They are not included in the headline platform read on page 1, which keeps the opening investor view clean and focused on the operating platform being offered today.

Regional opportunity note: organized tertiary and super-specialty penetration in the surrounding Eastern UP catchment remains shallower than in metros. That is why current ARPOB, payer mix, diagnostics capture and infrastructure readiness matter as much as absolute current occupancy.

8. Source notes and methodology

No.	External source note used in this pack
1	ICRA hospital industry outlook (Dec 2025 / Jul 2025): sample-set occupancy around 62%-64% in FY26-FY27; margins around 22%-24%; revenue growth 18%-20% expected in FY27.
2	CareEdge hospital sector note (Dec 2025): Indian hospital sector growth projected around 10%-12% CAGR over the next 2-3 years; EBITDA margins stabilised near 21%-22%.
3	Apollo Hospitals FY25 investor update (May 2025): Healthcare Services occupancy around 67%; ARPOB around Rs 63,569/day; EBITDA margin around 24.3%.
4	Listed hospital sector commentary (Dec 2025): listed hospital coverage discussed broadly around 20x-25x forward EV / EBITDA.
5	North India strategic transaction reference (Sep 2025): 150-bed hospital transaction implied roughly Rs 1.73 cr per bed.
6	NABH Hospital Accreditation Standards, 6th edition (Jan 2025): current external quality and governance framework reference.
7	ICRA bed addition note (Feb 2026): >34,000 beds expected to be added over FY26-FY30 with ~Rs 40,000 crore cumulative capex.

Methodology notes: (1) This version treats the owned primary campus as embedded in the integrated-platform valuation discussion rather than mechanically adding it back to EBITDA-multiple outputs. (2) The owned-campus reference is shown separately as downside support and for optional asset-light structuring. (3) FY23-24 is excluded from the normalized growth bridge because management indicated a one-off non-recurring project contribution in that year. (4) Internal pharmacy transfer values are disclosed, but no mechanical EBITDA elimination is forced in this teaser because the transfer-margin detail is not separately broken out here. (5) Current pack is discussion material only; final values remain subject to accounting, legal, tax, title, lease and commercial diligence.